MAJOR PROJECT · SCOPE

Prototype Scope

Consultation Readiness and Continuous Follow-up in Diabetes Care

Build window 13 October to 8 November 2026. Demonstration 28 November

Team Two people. No real patient data at any stage

1 What the scope is drawn against

The prototype puts weight on two claims, and everything in scope serves one of them. **Documents:** a value is shown to the clinician only if it can be traced to a region on the page it came from, and anything else goes to a person with the cropped image. **The weeks in between:** a patient records what happens between visits, by whatever route she can use, and it reaches her doctor without anyone typing it in.

One feature joins them, and it is why both halves are built thinly rather than one thoroughly: **the prescription the system has just read becomes her daily checklist.**

2 In scope

Capability	What it means in the prototype
Capture	Browser upload of a photographed document, one or several at a time
Normalise	Deskew, contrast, upscale. OpenCV, nothing clever
Detect	PaddleOCR detector with recognition disabled. Line boxes with coordinates
Read	A vision model reads the whole page. Output lines aligned to boxes by order
Verify	Drug list matching, a confidence floor, dose format rules. The deterministic layer
Route	Pass to the record, or to the confirm queue with a stated reason
Confirm	Queue interface. The crop renders before the controls enable. Keyboard driven
Printed lab reports	The same path with the recogniser switched back on. Eight analytes, values stored exactly as printed
Patient app	Installable web app on Android. Medicine ticks, glucose entry, the day's question, a short survey, meal photographs
Languages	English, Hindi and Urdu. Strings externalised to one file per language, translated by people
Other routes	Paper form entered by staff, and an inbound endpoint standing in for SMS
Record	Patient timeline in date order, and one page for the visit
Trace	Tap any value, see the original photograph with its box drawn on it
Audit	A row for every confirmation and correction. No viewer required
Compliance score	A stored score per patient per period, computed from medication ticks. Clinician view only. Every score reversible to the day array it

	came from
Evaluate	Gold set, annotation protocol, and the metrics script

3 Out of scope

Deferred, because four weeks is four weeks

Excluded	Why
Automated voice calls	Needs a telephony provider, an account and a budget. The data path is proven by the SMS endpoint
Forwarding rules and any notification	The highest scope risk in the project and the least necessary to prove
Canonical units, conversions and reportable ranges for lab values	A vocabulary problem that proves nothing about either claim. Values are stored and shown exactly as printed, with their provenance
A fourth and fifth language	The code cost is zero. The cost is eighty strings per language, reviewed by a speaker. Three is what two people can get reviewed in time
Offline capture queue	Cut outright to pay for the lab path and the patient surfaces. It was never a commitment
Caregiver accounts, consent screens, registration, authentication, printing	Seed four fictional patients. One hardcoded user. No login
Cross-document comparison	There will not be enough documents per patient for the result to mean anything

Excluded on principle, and still excluded in the finished system

Excluded	Why
Any estimate from a meal photograph: portion, calories, macronutrients	Interpretation, not capture, and the weakest model in the system. Leaving it out is also what lets the pilot promise that no patient photograph has to leave the clinic
Any score, total or trend over survey answers or free-text reports	Those are the patient's own words, stored as she gave them. The compliance score is computed from ticks, which are an objective record of an action, and from nothing else
The compliance score shown to the patient	It is a clinician-facing figure. Returning a grade to the patient is the interpretation the assistive boundary exists to prevent
Any diagnosis, treatment suggestion or clinical risk stratification	The assistive boundary the design rests on. Adherence counting is arithmetic over a recorded action; a clinical risk score is a judgement about a person's health

4 What finished means

Scope is only settled when done is defined. These eight, measured:

- A poor photograph of a handwritten prescription goes in, and drug names come out, each carrying a box on the original image
- At least one value lands in the confirm queue and a person settles it in under five seconds
- Every value on screen taps back to its crop on the original
- A prescription confirmed by a clinician appears as tick items in the patient app, and the ticks appear on the visit page
- A printed laboratory report goes through the same path and its values land on the same visit page, each carrying a box on the original

- The app runs end to end in Hindi and Urdu, and a day's question, a survey answer and a meal photograph recorded in the app appear on the visit page in date order
- A compliance score appears on the visit page, and tapping it opens the day array it was computed from, with every day traceable to a tick or a gap
- The evaluation script prints detection recall, read accuracy, false accept rate, false reject rate, and confirm queue items per document, **from a held-out slice of the gold set that was never developed against**

5 The cut list, decided now

	Cut, in this order	What survives it
1	Urdu	Hindi and English. The strings file is the deliverable, and a language can be added after the demonstration in an hour
2	Meal photographs	Ticks, glucose and the day's question already prove between-visit collection
3	The survey	Keep the single daily question. One question demonstrates the same path
4	The SMS endpoint	The paper route already proves a non-app channel reaches the same record
5	Printing	A workflow nicety, not a claim

Written in advance on purpose. Deciding what to drop at eleven at night the day before produces worse choices than deciding it now, calmly, when nothing is at stake.

SAY THE EXCLUSIONS OUT LOUD, AND JUDGE THE WHOLE SCOPE AGAINST ONE SENTENCE

At the demonstration, state what is built and what is designed but not built. A judge who finds a gap trusts nothing afterwards. Everything above exists to answer one question: when a machine reads a doctor's handwriting and a deterministic layer checks it, how much reaches the clinician as fact, how much reaches a person, and is the second number small enough to live with?